

Hope for Vulnerable Children Association (HOVUCA)



Research on barriers faced by adolescent girls and young women (AGYW) in accessing HIV and sexual and reproductive health services

Research Organisation	Hope for Vulnerable Children Association (HOVUCA)
Research Coordination & Project Leadership	Lum Evelyne — Project Coordinator, HOVUCA
Study Population	Adolescent girls and young women aged 14–24 years
Study Location	Yaoundé III Municipality and surrounding neighbourhoods, Cameroon
Research Support / Partnership	Y+Global, through the HER Voice Fund

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- **Research coordination and project leadership:** Lum Evelyne, Project Coordinator, HOVUCA, who coordinated and led the research process on behalf of HOVUCA.

Research at a Glance

- 302 adolescent girls and young women participated in the quantitative survey.
- 8 key informant interviews were conducted with community, education, health and decision-making stakeholders.
- 4 focus group discussions were conducted with young people.
- The study examined knowledge, availability, accessibility, affordability, stigma, confidentiality, human rights and policy-related barriers.

Yaounde, Cameroon. September, 2020

ACRONYMS	Full Description
AGYW	Adolescent Girls and Young Women
AIDS	Acquired Immunodeficiency Syndrome.
FGD	Focus Group Discussion
FGM	Female Genital Mutilation
HIV	Human Immunodeficiency Viruses
HOVUCA	Hope for Vulnerable Children Association
IEC	Information, Education and Communication
KII	Key Informant Interview
NGO	Non- Governmental Organization
PEP	Post Exposure Prophylaxis
SDG	Sustainable Development Goals
SRH	Sexual and Reproductive Health
SRHR	Sexual and Reproductive Health Rights
STI	Sexually Transmissible Infections

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1. INTRODUCTION

Adolescents and Young women (AGYW) 14 - 24 represent a significant proportion of Cameroon's population. However, for many African youth, the transition through adolescence continues to be fraught with several challenges and difficulties. These include high rates of poverty, illiteracy, unemployment and underemployment, violence, sexual coercion and exploitation, substance abuse and other deviant social behaviors. Coupled with high rates of judgmental values by adults and negative societal/cultural attitudes which deny young people access to appropriate information and services especially on sexual and reproductive health, it is not surprising that AGYW in some Africa Countries including Cameroon currently have some of the highest rates of HIV/AIDS. It is estimated that more than half of all new HIV infections are among young women and girls while between one quarter and one half of adolescent girls become mothers before they turn 18. Adolescent girls are two to five times more likely to die during pregnancy or childbirth than women in their twenties

More than half of the Cameroon's population is under the age of 25. While young people face precarious situations given the high unemployment rate, AGYW suffer disproportionately from negative sexual and reproductive health outcomes, which challenge their ability to contribute to their communities' and countries' development. AGYW face social and economic barriers in accessing sexual and reproductive health information and services, which is evidenced by persistently high levels of unmet need for contraception, maternal mortality, and HIV incidence. Young women aged 14 to 19 are twice as likely to die in childbirth as adult women, and half of all new HIV infections occur in AGYW 15 and 24. Such negative sexual and reproductive health outcomes have economic, social, and health consequences that affect them throughout their lives, as well as their families, countries, and the global community.

This unique research carried out by young people (young women and young men) is essential reading for policy makers and programme designers seeking to improve HIV/SRH for AGYW in Cameroon and elsewhere.

This research funded by Global Network of Young People Living with HIV titled *“research on barriers faced by AGYW in accessing HIV/SRH services”* was set out for the following objective.

2. OBJECTIVE OF THE RESEARCH

The overall objective of this study was to gather stories from 300 AGYW through questionnaires and focus groups on the problems they face in upholding their HIV/SRHR and also to listen to stakeholders through key informant interview what they have in hand to handle the health issues of young people and hence document gaps that will aid in advocating for better policy reforms that will meet the health needs of AGYW and all young people.

3. METHODOLOGY

The research was conducted in the Yaounde III Municipality and its neighborhood. The target population for research consisted of AGYW (14-24 years), community leaders, principals and teachers, health personnel, decision makers. The questionnaires were coded for data entry. The answered questionnaires were entered using the Magpi App, then exported to excel for analysis. The information from the Focus Group Discussion (FGD) and key informant Interview (KII) were transcribed and analyzed for contents.

3.1. Questionnaire

A self-administered, quantitative questionnaire was used for the survey. There were a total of 70 questions administered to 302 AGYW 14 – 24 years. The questionnaire was divided into two sections (SRH and HIV). Both sections had sub-sections although with a common demography. Section 1– SRH had three sub sections which included knowledge on SRH, family support system and family planning services, religion and traditional practices and human rights. Section 2 – HIV also had 4 sub section which included: Knowledge on HIV related issues, availability, accessibility and affordability; Stigma and discrimination, human rights and confidentiality and lastly policies and human rights. All two section had a common sub section which was the socio demographic data of the respondents.

3.2. Key Informant Interview (KII)

Eight (8) key informant interviews were conducted through face-to-face meetings. A semi-structured interview guide was used. Participants of key informant interviews included community leaders, principals and teachers, health personnel, policy/decision makers and parents

3.3. Focus Group Discussion (FGD)

A total of 4 focus group discussions (FGD) were conducted among community youth to gather information on the perceptions of youth friendly services and barriers that limit their access to SRH services provided in the health facilities within their various areas of resident. The FGD guide was used because of the nature of the research, the need to develop themes and also because they provide opportunity for probing. The guide contained an introduction of the researcher and full disclosure of the research, ethical considerations as well as request for consent. This was followed by a 10- question guide on issues relating to their knowledge about sexual and reproductive health problems of youth and particularly AGYW in their communities, accessibility and affordability of health facilities, health workers' attitude, and the mode of information transmission within their communities

KEY Findings

Profile of Respondents

The age of respondents ranged from 14 to 25 years. Majority (58.6%) of the respondents fell between the ages of 14 to 19 years. 96.3% of respondents have attended school and can read a newspaper. Respondents 32.7% have completed high school. Majority of respondents were Christians 92.3% and many of the respondents (73.6%) are unemployed.

The following barriers for AGYW in accessing SRH Service were Identified through the research

1. Knowledge on Sexual and Reproductive Health

1.1. Puberty and menstruation

Respondents (42.2%) 14-18 years know to a certain level what is puberty and some of the changes that occur during puberty. 72% of respondents seek services with the changes that occur on their bodies with their mother and only 8% seek services with their father meaning an adolescent girl lives only with the father is bound to have some major SRH problems. FGD respondents identified peers, parents, teachers, radio and television as the main sources of information on reproductive health. However, most of the adolescents especially those out of school relied mostly on their peers for information on SRH. Findings from FGDs showed that parents were an important source of information. The findings from FGDs also showed that for in-school adolescents, teachers emerged as another important source of information on reproductive health. 70% of respondents have ever heard of menstruation before starting to menstruate from their mother, sister or aunt and never from their father, brother or uncle. 97% of respondents are menstruating. However, 53.3% of the respondents do not know how to calculate their menstrual cycle which can help them prevent unwanted pregnancies.

Findings from key informant interview reveals that at some point there is a dependency notion that creates a vacuum and as such becomes a big barrier to especially adolescent girls to access HIV/SRH knowledge and services. This was clearly shown in responses given by key informants when responding to the question on what role they play in ensuring that AGYW have comprehensive knowledge on HIV/SRH and how they are helping them to access services with easy. The following quotes is what they expressed:

".....sometimes I do not really bother much to discuss sexuality with my daughter because I know they teach them all those things in school.....girl children feel more comfortable discussing these issues with their female teachers and other people out of the family circle because there is an in-built cultural belief that if a child discusses such things with their mother or father it shows they are prostitutes or they lack respect for their parents.....that does not mean I do not discuss certain health related issues with my daughter at all....." (Parent (female), Key Informant)

In another excerpt....

".....well, we have club periods where sometimes we can seek the help of health personnel or sometimes some NGOs come to give health talks with students including the adolescent girls, but the time allocated for clubs so limited that they cannot really learn much.....especially for the girl child, their mothers are solely responsible to educate them on such issues especially as they will have to tackle particular female problems that the girl will have. we are only supporting" (School head, Key Informant)

This clearly elucidate the deadlock position where in-school adolescent girls find themselves talk less of the out-of-school girls who depend mostly on their peers for knowledge and advice (good or bad) on SRH issues.

1.2.Family support system and family planning services

Adolescent girls (76.8%) have discussed sexuality with their mothers and only 20.1% have had such discussion with their father. 2.9% have never discussed such topics with neither the mother nor father. A participant in a FGD has this to say:

"most of us actually prefer to get information about our bodies from our friends and sometimes from nurses if we have them around. I really feel comfortable discussing about this with my friends" (adolescent girl, FGD, out-of-school)

Majority 72.8% of the target group who have heard about contraception are young women 18-25 years and 28.3% adolescent girls 14 – 17 years have not heard about contraceptives. Of the young women 18 – 25 who have heard of contraceptives only 24.8% have ever visited a health facility or doctor of any kind to receive services or information on contraception, pregnancy, abortion or STIs.

Those who answered YES to have visited a health facility or doctor for any of the reproductive health issues, only 18.2% were well received by the facility or doctor they visited. On the question on availability of IEC materials on contraceptive at a health facility, only 19.2% of respondents have ever seen a poster of contraceptive or condom and only 13.2% have so far received a brochure on contraception from a facility. Only 14.2% of respondents have attended a talk on contraception.

In the FGD, respondents have heard about conception, some could name the different methods but lack information or education on how to use them and this lack of knowledge on usage and side effects presents them from demanding and using the products for fear of the unknown. Some respondents get such information sometimes in a disorderly manner and from ordinary people in the quarters which gives no assurance and consequently this deters them from desiring the services. The following is an excerpt from a FGD respondent.

"I heard from a friend and even my neighbor that there is something called a coil that can be attached to the baby and it will prevent unwanted pregnancy. I have never heard of this during clinic lectures that I attended for my two kids. However, I started having interest because I did not want to have more children but when I discussed the issue with my friend she said I should not try, that the "thing has a lot of side effects. This just killed my spirit and I was scared and I abandon the idea"(Young women, FGD)

On knowledge regarding the different types of contraception and use, majority 66.3% have heard about condoms, pills and withdrawal. Also, on the question on which contraceptives is most suitable for young people, condoms 43.7% was the most suitable followed by periodic abstinence 22.1% and 5.9% were for injection and withdrawal. On usage, condom use is 38.4% followed by periodic abstinence 17.8%. Only 10.9% of respondents have dare to ask for contraceptive services during consultation. On education on pregnancy and STIs by health workers, only 19.5% of respondents confirm having received education on STIs from a nurse or doctor while 12.9% have received on pregnancy and only 17.5% could dare to ask questions on this topics because of fear and judgment as one respondent in the FGD expressed.

About seeking family planning services from others sources other than hospitals, majority 66.2% answered yes and majority 55.1% seek these services from drug store and pharmacy. Concerning knowledge on condom, 81.7% have seen at least a male condom while 38.7% confirmed using condom. Many (47.6%) do not use condom. With regards to the question on what condoms can be best used for, 46.6% respondents said condoms are best for preventing pregnancy while 43% said that condoms are useful for preventing HIV. 40.7% respondents agreed they would feel embarrassed when going to buy or obtain condom, 32.1% disagreed with the issue of feeling embarrassed, 22.1% could not be definite whether it would be embarrassing or not and 4.9% stood neutral on the issues.

Responding to the question whether there are some SRH services that can be denied because they are very young, majority 54.3% answered Yes and 37% of respondents quoted abortion as an example of a SRH service denied people because they are very young.

1.3 Religion and traditional Practices

As previously stated 92.3% of respondents were Christians hence, 87.4% of respondents said religion is a barrier to using condom and contraceptives and doing abortion. For things that happen in the community and harmful traditional practices for women and girls, 74.6% of respondent quoted forced marriage, Female Genital Mutilation (FGM), rape and sexual exploitation. Responding to the reasons why the community practice FGM, majority 63.9% said it is for the purpose of maintaining culture and tradition. In the FGD, respondents also said culture and religion pose a huge problem in accessing SRHR as they shared experiences they have gone through due to stigma from the religious community.

In the FGD, it was shown that barriers to contraception use among Muslim young women included a lack of basic reproductive knowledge, insufficient knowledge about contraception, misconceptions and myth, and negative attitudes. Women had negative attitudes towards family planning for limiting the number of children but not for child spacing, which reflected religious views towards family planning. In a Key Informant interview, we sought to know how far tradition has gone to ameliorate norms so that women and girls can access SRH services without judgment from family or community, the following quote was a response from a traditional leader.

....."you have to understand that wiping out some of the cultural norms is not very easy as we think, it has to be a gradual process..... I agree with you that mentioning abortion is a cultural taboo especially in the rural communities but discussions on eliminating such believes have been on for like more than 2 decades, in some communities some changes have occurred but majority are still holding on the traditionaltake for example

female genital mutilation that happened rampantly in the South West Region of Cameroon. Today, it is not more the case, people may still be performing it but that is in hiding and eventually it will be wiped out. We are working and those changes will come" (traditional head, Key Informant)

The following barriers for AGYW in accessing HIV Services were Identified through the research

2. HIV and STIs

2.1. Knowledge on HIV and STIs

Respondents 95% have heard of HIV and AIDS, 47.3% know HIV and AIDS cannot be cured. 41.3% said a person with AIDS always look emaciated or unhealthy. 63.5% respondents know that they can take a test to find out if they have HIV. However, 87.3% draw conclusion of their status only from the first test and this is dangerous as they are missing information of the window period which puts them more at risk. Respondents (50%) know they can protect themselves from getting HIV through abstinence, use of condom, faithfulness of both partners, avoid sharing razors and avoiding unsafe blood transfusion. This is superficial as they are unable to put all this measures in practice due to fear, shame and stigma of buying or obtaining condom, this fear of shame was clearly articulated by respondents in the FGD. Respondents (84.5%) have never heard about Post Exposure Prophylaxis (PEP). As regard the question on what treatment a woman who experience forced sex should have after such an incident, only 22.8% respondents mentioned HIV Post exposure prophylaxis. Only 19.5% respondents could guess where they can obtain post exposure prophylaxis.

In the FGD, respondents had some knowledge of STIs and could even list some like – syphilis, HIV, chlamydia, gonorrhoea, Pelvic Inflammatory Disease and Genital Herpes. More than 70% of respondents in the FGD did not know how one gets Hepatitis B, some have heard there is a vaccine for Hepatitis B but do not take it serious because they lack appropriate information on it.

Findings from KII showed that government has put in place a policy to create awareness on HIV and disseminate information about HIV to adolescent and youths and has mandated some ministries to get involve and implement these policies. The following statement from a key informant explains this.

".....the government has a policy to make sure young people are well informed of the pandemicfor example in the national strategic plan to fight against HIV and AIDS in section 4.2.3 – Education and youth sub sector, the ministries of Basic Education, Secondary education, Youth and Civic Education and Higher Education etc. have been mandated to integrate information and education on HIV / AIDS into the curricula for the training of students at all levels and interventions for teachers. Similarly, the Ministry of Youth and Civic Education will coordinate interventions that target youths that have dropped out of school and those that have never been to schoolit is left for this sectors to do their work" (Health Officer, Key Informant)

2.2. Availability, accessibility and affordability

42% of respondents know of a facility offering HIV testing closer to where they live and as concerns distance from them, 13.9% respondents said 1 – 2km, 7.6% said 2 – 3km, 2.6% said 3 – 4km, 8.6% said 4-5km and 58.9% of respondents could not estimate the distance due to long distance. Majority of respondents 68.2 % do not have the financial means to pay for services. A respondent in the FGD had this to say

".....living cost in this Yaounde is first of all very high, I do not know any hospital or clinic here in Yaounde that I can go there and consult for free before struggling to buy the drugs they will prescribe, not to talk of the numerous tests they will asked you to do and specify their own laboratory where you have to go and do it.is not easysometimes am just scared to go to the hospital and I prefer to seek common herbs in the quarter" (young women, FGD)

The findings showed that in urban settings there are many facilities and accessible though not relatively affordable. However, in the FGD, experiences with regards to availability, accessibility and affordability shared were not the best. The following is an excerpt from a FGD respondent

".....we should not look at this problem of HIV only in Yaounde here where there are many facilities.....we should think about those in the villages. In my village which is big, there is no functional health centre..... when am in the village and need health services, I have to go but to a near village which is very far and to see a motorbike going there is not easy and even if you see the motorbike only the amount of money they tax, you will not be able pay and the health clinic will not consult you for free..... pregnant women are force to deliver at home with the aid of traditional mid-wives, if you survive fine for you, if you die bad for you!" (young women, FGD)

A respondent of the key informant interview reaffirms the availability and accessibility of health facilities that cater for the needs of young people in general. The following is a quote from the key informant:

"We have service-points for adolescent reproductive health service in this district where we provide education, counselling, contraceptive service to adolescent who visit us.....young women come to us most often for STIs unfortunately young women hardly approach us for abortion care but the services are availability" (medical practitioner, Key Informant)

2.3. Stigma and discrimination, human rights and confidentiality

Respondents (54.3%) said they have been tested of HIV and 40% have not done the test and answering to the question on the reasons they have not been able to go for the HIV test, respondents advanced the following reasons in order of gravity – I don't really know where to go for testing 92.3%, fear of meeting people I know 89%, fear from facing results 76.4%. Majority of respondents (67.7%) consider keeping one's HIV test results confidential is very important.

In a FGD, some respondents said they do not feel comfortable going to a health facility around where they live because their results might be spread by the health workers who know them personally, a participant had this to say:

"I do not like to consult in a clinic that is near my house, those nurses talk too much, they are always pointing people when they are walking pass and saying something, I prefer to pay my transport and go to another clinic where am not familiar with the health workers there" (young women, FGD)

As concern having other STIs beside HIV, 76.4% respondents confirm they have been victims of other STIs and for knowledge on sign and symptoms when a women is infected with STI, 64.5% mentioned pain during urination, 61.2% quoted vaginal discharge and 42.3% talked about ulcers/sores in genital area. For where to obtain treatment for STI, respondents responses goes thus in descending order – 81.7% government hospital, 60.9% health centers/clinics, 44.7% pharmacy and 9.6% traditional doctors.

Findings from KII reviewed that public health workers are often given refresher courses on medical ethics especially lessons on confidentiality and non-stigmatized services. However, respondents of the quantitative questionnaires (only 31%) believe health workers keep people's HIV test results confidential and do not reveal them to anyone outside the health clinic. Examples of such unethical behaviors from health service providers repeatedly came up during the different FGDs. Respondents (61.2%) said it is not okay to tell other people that someone else has HIV.

2.4. Policies and Human Rights

Majority 93.7% do not know any policy on HIV/SRH although a respondent of a Key Informant Interview (KII) mentioned there are documented policies on HIV/SRH. Respondents 43.2% know their SRHR though not able to exercise their rights nor access services. A respondent of the KII in trying to respond to the question on availability policies and how far the policies are being implemented he had this to say:

".....the government has policy documents on HIV/SRH, we have policy document like - The National strategic plan for the fight against AIDS 2018 - 2022 and the National Guidelines on the Prevention and Management of HIV in Cameroon.....this are the policies and the government has made efforts to provide facilities and HIV/SRH services if not in all locations of country, at least these service are available in the different urban areas of the country. While the government is making efforts to reach the population with information and services, clients also have an obligation to move towards facilities and demand for information and the services" (Public Health medical practitioner, Key Informant).

Respondents of FGDs (more than 50%) age 14 – 17 did not know exactly their SRHR, they have never received education on the SRHR neither in school nor at home or even in a health facility. Some were hearing the concept for the first time. However, respondents of FGDs (70%) aged 18 – 25 had knowledge of some of the rights and could name some like – right to health care, education/information, protection, non-discrimination, participation etc

CONCLUSION

This research has indicated that there is an array of barriers inhibiting AGYW from accessing HIV, sexual and reproductive health and STI services. These barriers include both the receiver and supplier factors and linked so far to knowledge and behavior of young people and particularly AGYW, communities and service providers, health coverage and poverty.

Lack of awareness, inadequate information, attitude of health care workers and other significant barriers posed by the current state of most SRH services are allegedly unwelcoming to AGYW clients.

Religious and traditional norms have been identified as barriers to contraception use and access to SRH services and education. Family support system and the community have a substantial bearing on women's contraceptive use and access to SRH services. Husband and family opposition played a major role in contraception access and use. Fear of stigmatization and being labelled as having pre-marital sexual relations among young unmarried women acted as the main barrier to accessing contraception and seeking SRH information and services.

There is a general awareness of the existences of health facilities that offer HIV/AIDS, SRH and STI services but there is a low awareness of where specific services are offered. Lack of detailed knowledge of SRH issues poses high risks as we previously saw in the findings, a greater number AGYW have a basic knowledge of family planning, HIV/AIDS and STIs but only a few have in-depth, detailed knowledge on this health issues. This brings us to questioning the quality of information being disseminated and/or the effectiveness of the dissemination methods.

Taking into consideration that AGYW shy away from using HIV/SRH services, strategized method have to be put in place to entice and retain the AGYW clients. AGYW respondents have expressed a multiplicity barriers to service use, which involves legislations and guidelines that may restrict their access to affordable services and useful information, feeling of shame of being seen at health facility, fear that confidentiality will not be respected, and a worry that health workers will be aggressive and judgmental. Other barriers articulated included operating times, lack of transportation, and high cost of services. There is need to find practical ways to assess the current operations of primary health care facilities and take steps to ensure that they are “youth friendly. Legislations

RECOMMENDATIONS

- 1) Governments particularly that of Cameroon should reinforce by mandating comprehensive HIV and sexuality education in schools which is linked to integrated sexual and reproductive health services. They should ensure services reach all young people, especially AGYW in both in- and out-of-school. This is vital to preventing unwanted pregnancy, HIV and other STIs, and to achieve SDG 3 target 7.
- 2) Development and production of a health facility – service provision diary that clearly indicates the following: which facility is offering which specific HIV/SRH/STI services, the distance, some basic information on financial commitment required, service hours. This document should be disseminated to all institutions or arenas where young people particularly AGYW could be found so that they can easily access it. This will help to solve the problem of lack of awareness of which health facility offer what services.
- 3) In Cameroon like in some other countries abortion is illegal. Therefore governments should ensure AGYW have access to effective contraception and work towards safe, legal abortion care. Young women who are poor or in other vulnerable circumstances are at high risk for unsafe abortion, yet this is entirely preventable through access to contraception and safe abortion care for, the two methods are essential and cost-effective components of any strategy to reduce maternal mortality and achieve SDG 3 target 1.
- 4) Provision should be made for universal access to youth friendly and nonjudgmental HIV/SRH services. Services must respect the rights to confidentiality, privacy and informed consent.
- 5) Government and ministry of health in particular and other NGOs be they national or international who are mandated to the course of HIV prevention should make *Post Exposure Prophylaxis* (PEP) available and bring it

closer so that AGYW who are disproportionately affected by sexual violence should have easy access. They should also make available services to prevent sexual violence and provide care to those affected.

- 6) Government and policy makers should take actions to reject laws that criminalize abortion. This should be accompanied by a rejection of patriarchy laws and deep-rooted stereotypes and widespread church dogmas against condoms and abortion. Legalizing abortion is a pre-requisite to delivering high quality safe abortion services and reducing high death rates among AGYW. In fact not legalizing abortion is an invisible way of legalizing unsafe abortion thereby increasing the death toll for poor and vulnerable young women. AGYW must be given the leeway to exercise their sexual and reproductive health rights.